



Haven Paediatrics

Engagement brief for Abigail

Prepared by Dr Debo Odulana, Consult for Africa · Internal, not for the client · August 2026

Welcome to Haven. You are joining at the hinge point: the audit has landed with the board, and the transformation work has not started. This brief is what you need to be useful in the first week. Read section 3 before you speak to anyone at Haven or write a word that could reach them. On this engagement, how we carry ourselves matters as much as what we find.

1. Where the project actually is

One sentence: we have told a young hospital that its care is excellent and the systems underneath it are not, and we are now waiting on the founders before we build the fix.

Stage	Status	What it means for you
Diagnostic audit	Complete	Four evidence streams, reported at 101 pages plus a 21-slide board deck
Report delivered	Sent 30 Aug 2026	Five individual emails to the founders, cc Debo. They also had it on WhatsApp first
Founders' survey	Live, awaiting responses	Your first live task. Nobody has completed it yet
Strategy session	Not yet scheduled	Cannot be designed until the survey is in. The survey is what makes it worth holding
Transformation programme	Not started	Scoped in the report as five decisions and a 90-day plan. Waits on the session

The critical path runs through five people answering a twenty minute survey. Until that happens the strategy session cannot be designed and the transformation cannot start. Treat chasing it as real work, not admin.

2. What Haven is

A private paediatric facility in GRA Ikeja, about eighteen months old. Five general paediatric beds and a three-bed neonatal unit. It funds its own salaries, holds a real local reputation, and is scaling into more complex care, which is exactly when the systems beneath a facility have to become deliberate rather than assumed.

The five owners

Kabir Aregbesola, Mrs Abisodun Alli, Dr Shakirah Saliu, Dr Odedina (Medical Director), Ogochukwu Odum. Use those names and spellings exactly; several have been written wrongly in the past and corrected. Dr Shakirah Saliu is a former co-founder of Debo's from Doctoor Health, which is why the relationship is close and why the tone of everything we send matters more than usual.

The operating reality you should know cold

Measure	Position	Why it matters
Patient experience	4.60 of 5, 16 caregivers	13 of 15 would definitely recommend. This is the asset the whole plan protects
Staff safety grade	22 of 23 very good or excellent	Staff rate the care they give highly, anonymously, with nothing to gain



Measure	Position	Why it matters
Staff tenure	12 of 16 under a year, none beyond two	The multiplier on every other problem. Nothing compounds
Organisational maturity	2.5 of 5, down from 3.0	Patient-facing workstreams rose. Everything supporting them fell
Registered nurses in H1	Four	Eight days in July had two people covering a 24-hour hospital
Tariff lines priced at zero	1,952 of 2,413	Four fifths of the price list carries no price



3. Read this before anything else

Four rules govern this engagement. They are not style preferences. Three of them exist because a consultant's written document is discoverable and can end up in front of a regulator, a claimant or a journalist.

3.1 The incident, and how we hold it

In May 2026 Haven lost a child. The child was referred in already critically ill and may not have been saveable under any circumstances. The loss is not attributable to the facility. It is what prompted leadership to call us.

- **We do not raise it. Ever.** Not in writing, not in conversation, not in a WhatsApp message. If a staff member raises it, listen with care, do not probe for blame, and move on.
- **Every client-facing word is forward-looking.** We are here because a strong young facility is putting proper systems under its growth. Not because something went wrong.
- **Why it is this strict.** A written record linking a death to any facility gap is a liability for Haven and for us, and it reopens a wound for a grieving board. Every finding we have stands on its own without ever invoking the loss.

3.2 The reporting question, and how we frame it

This is the newest and most delicate thing on the engagement, and it did not exist when the last brief was written. The audit found that Haven's board reporting does not reconcile to its own transaction record at monthly level: January overstated by N8.00M, March understated by N7.87M, while the half-year total lands within 0.14 percent of correct.

The report deliberately does not draw a conclusion from this, and neither do you. It states the facts, notes that each item has an innocent explanation available, says the pattern of them together needs one, and recommends an independent reconciliation done outside the operations function. Procedural, never accusatory.

- **Individuals are referred to by role, never by name.** The Head of Operations. A pharmacy technician. The evidence packs name people; our client documents do not.
- **Do not harden the language.** Not in a slide, not in an email, not verbally in a meeting. If someone at Haven presses you for a view on it, the answer is that the reconciliation will settle it and we are not going to prejudge it.
- **If this escalates, it goes to Debo.** Immediately and only.

3.3 The relationship and governance

- Debo was **invited to Haven's board from inception but does not formally sit on it.** Say invited to the board, never sits on the board. The board is not yet formally constituted.
- CFA is a paid partner to a company Debo is close to. A related-party situation handled with total transparency: pricing disclosed to all owners, clean fixed fees, no success fee. You do not manage this, but know it, and never imply special treatment or leverage.

3.4 The commercials

The fee is N9.3M, negotiated down from a standard N17M. A large concession has already been made. Do not use the discount as leverage or discuss it as a favour. If payment comes up in any form, it comes to Debo.

4. What the audit found

The thesis in one line: the care is ahead of the systems holding it up. Haven is not underperforming; it is a hospital whose performance rests on particular people rather than on systems that would survive their leaving.

The three things the board has to close first



	Finding	Where it stands
0 1	Regulatory currency. Monthly reports recorded the facility as unregistered every month January to April, when registration was in fact held.	Largely closed. Registration confirmed by Debo; this year's renewal pending. Survives as a reporting finding, not a licensing one
0 2	The establishment does not match the hospital. Four nurses through the half year, no pharmacist rostered on any day, eight days with two people covering 24 hours.	Open. The most pressing of the three and the most straightforward to fix
0 3	The revenue record is a nursing handover sheet. The nursing procedure has nurses filling the revenue spreadsheets from memory at end of shift.	Open. Fix is to let the clinical record system be the record and retire the manual layer

Underneath those

- **People systems rate 1.4 of 5**, the lowest score anywhere in the work. No performance management, no structured induction, no training calendar, no exit interviews. Staff were told to write their own indicators.
- **Processes exist and are not followed**, for four reasons: nobody was taught them, nobody measures them, the workforce is mostly new, and several cannot be followed as written because they assign work to posts that do not exist.
- **Owner alignment is unresolved**. Two legitimate instincts, one weighted to protecting staff and one to commercial discipline, never reconciled into a rule. This is what the strategy session exists to settle.
- **The commercial gap is yield, not demand**. The neonatal unit and the laboratory are the two highest-value assets and neither is managed for yield.



5. A methodological lesson, learned the hard way

Read this one carefully, because it will save you from the mistake I made twice.

Much of the documentary analysis was built from evidence packs that catalogued 74 findings across 34 client documents. Two of those findings were wrong, and both failed the same way: the pack read **absent from the documents supplied** as **does not exist in reality**.

The claim	The truth	How it surfaced
No consultant rota exists	A consultant rota does exist. It simply was not among the documents supplied	Debo corrected it from direct knowledge after reading the draft
The facility is unlicensed	Registration is held. Only this year's renewal is pending	Debo corrected it from direct knowledge after reading the draft

Any finding of the form *X does not exist* is a claim about the world built from a gap in a folder. Before it goes in front of a client, either confirm it with someone who would know, or write it as what it actually is: we asked for X and did not receive it.

Both corrections were absorbed cleanly because they came before the report went out. The cost of the same error after delivery would have been the credibility of everything else in a 101-page document. That is the asymmetry to hold in mind.

6. What happens next, and your part in it

Step	What it involves	Owner
Chase the founders' survey	Five founders, twenty minutes each, live now. Completion is visible on the admin page and Debo gets an email on each submission. Chase warmly and individually, never as a group message	You, with Debo on the relationship
Synthesise the five	The instrument is built on forced trade-offs, constant-sum allocations and a section asking each founder what they think the others believe. The divergence is the output, not the average	You draft, Debo edits
Circulate the pattern before the session	So nobody walks in cold and nobody is ambushed by a finding about their own position	Debo sends
Design and run the strategy session	Built around the five decisions in the report: growth axis, operating model, workforce proposition, service portfolio, funding	Joint
Scope the transformation programme	The 90-day plan and the initiative register are already written. They need sequencing against whatever the board actually decides	Joint

7. How we write here

Client documents are partner-level counsel to founders Debo knows personally, not audit papers written for a file. The register matters as much as the content.

- **Write to them, not about them.** Your nurses, your tariff, what we found, our judgement. Third-person audit prose puts distance between us and a client who invited us in.
- **Never let a systems finding read as a personal one.** Name people by role, and warmly. Your business development team, which is working genuinely hard.
- **Say what you cannot tell them.** Candour about the limits of the evidence reads as integrity and protects the firm.



- **No em dashes anywhere.** House rule, applies to every document, email and slide. Use a comma, a full stop or a colon.
- **Do not name our platforms in client body copy.** Describe what is done, not the product that does it.
- **Short paragraphs, one idea each.** Debo's register is declarative. State the position and stop. No rhetorical build, no sentimentality, no performed humility.



8. Where everything lives

Artefact	Path	Note
Audit report	docs/haven-organisational-audit-report-cfa.pdf	101pp. Built by scripts/build-haven-audit-report.py
Board deck	docs/haven-audit-findings-deck-cfa.pdf	21 landscape slides, same numbers as the report
Survey results	docs/haven-survey-results-cfa.pdf	Staff and caregiver surveys in full
Founders' survey	consultforafrica.com/haven-leadership-survey.html	Live. Responses land in AuditSurveyResponse
Live results page	/admin/haven-survey	Completion tracker, allocations, tensions, perception gaps
Survey analysis cache	docs/data/haven-survey-analysis.json	So the report and deck rebuild without database access
Evidence packs	~/Downloads/HAVEN_PAEDIATRIC_AUDIT_DATA_PACK*.md	74 findings from 34 client documents. Read section 5 first
Send script	scripts/send-haven-audit-report.ts	Dry-run by default. Never Zoho SMTP, ZeptoMail HTTP API only

9. Do and do not

Do	Do not
Frame everything forward: building systems for growth	Reference the May 2026 loss, in writing or in speech
Say Debo was invited to the board	Say or imply he sits on the board
Keep the reporting finding procedural and role-based	Name an individual, or offer a view on what the pattern means
Write <i>we asked for X and did not receive it</i>	Write <i>X does not exist</i> without confirming it
Report survey results as grouped totals	Let management see any individual staff response
Chase the founders one by one, warmly	Send a group chaser that lets four people wait for the fifth
Route money, board and incident matters to Debo	Negotiate price or discuss the discount yourself
Lead with: the care is ahead of the systems	Sell the work as a set of process fixes

Welcome aboard. The thing that makes this engagement unusual is that the client is good at the hard part and weak at the easy part, which is a far better problem than the reverse and a much more satisfying one to fix. Ask me early rather than late, on anything at all.

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